

EXECUTIVE BRIEFING

Medicare Spending & Efficiency Benchmark

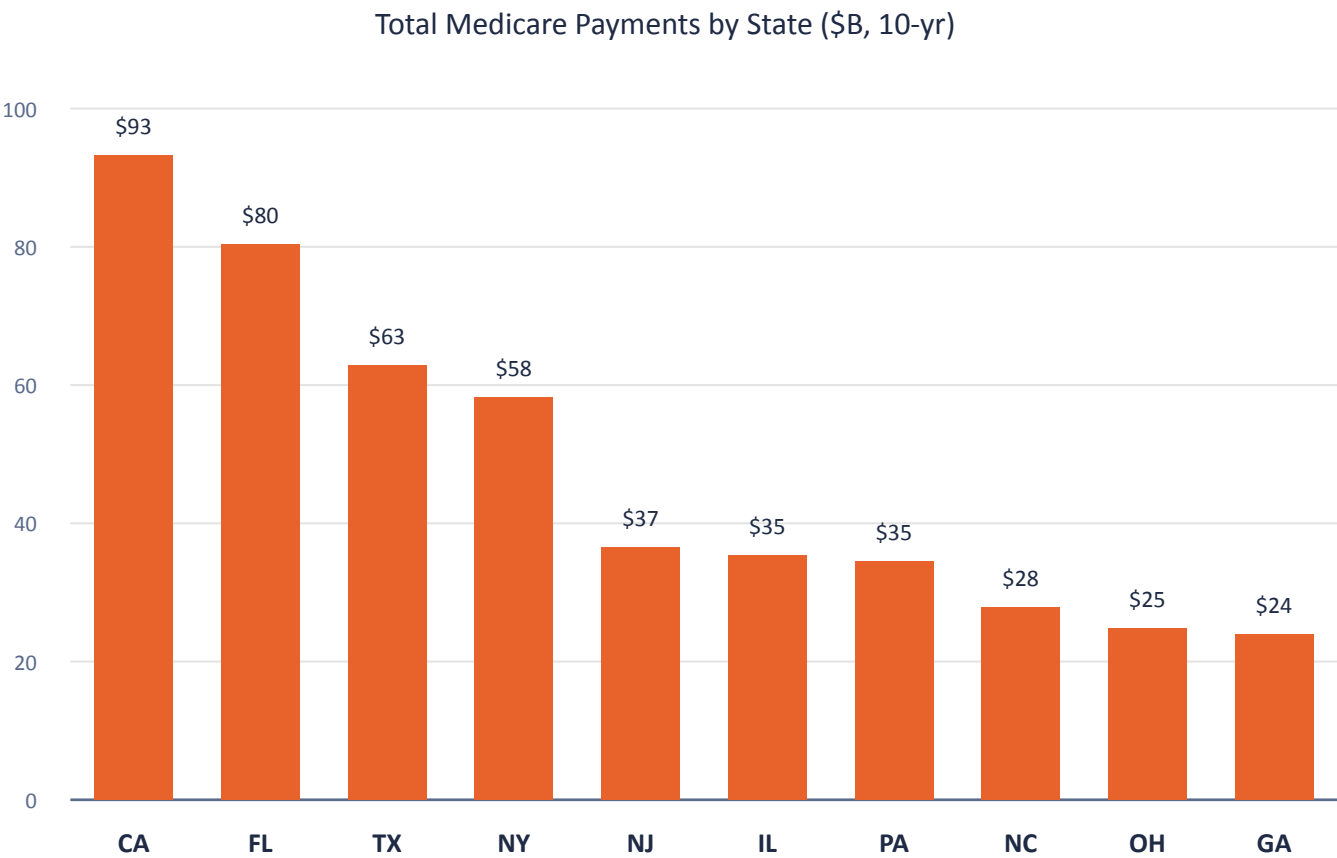
A state-level analysis of spending concentration, cost drivers, provider workload, and operational strain

Scope: Top 10 U.S. states by Medicare payment (56% of the \$853.6B national total) | CMS Medicare data, 2014–2023 | Metrics: total payments, service volume, cost per service, provider capacity

SPENDING CONCENTRATION

Which states consume the highest share of Medicare spending?

Four States Account for 34.5% of National Medicare Spending



34.5%

of the ENTIRE \$853.6B national Medicare total sits in just 4 states — CA, FL, TX, NY

(these 10 states combined make up 56% of the national total)

CA alone represents 10.9% of ALL national Medicare spending — roughly 1 in 9 national dollars (\$93.2B of \$853.6B). Spending share is concentrated, not evenly spread — CA and FL together still outspend the five smallest states shown combined (\$173.6B vs. \$146.4B).

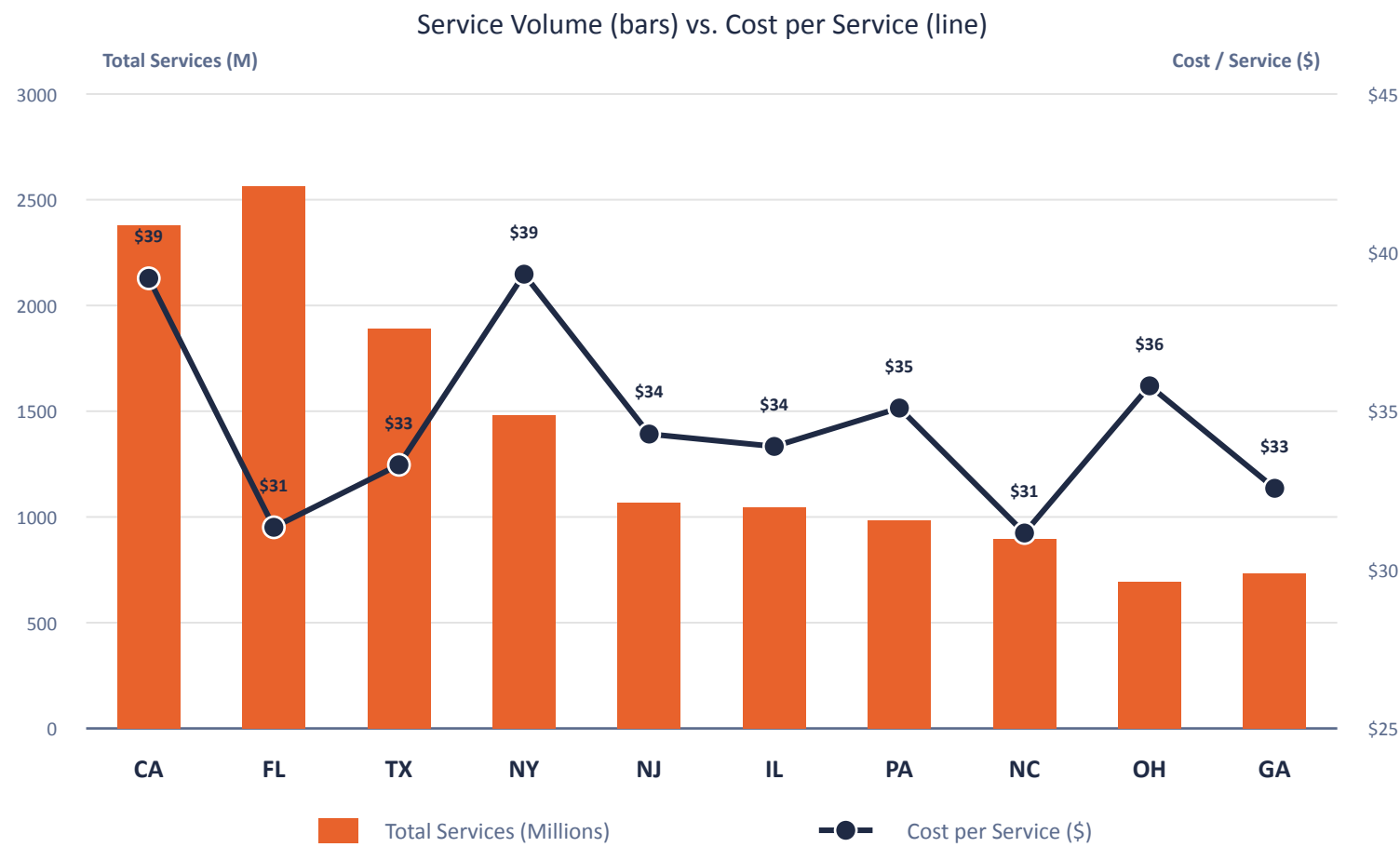


Recommendation: Prioritize utilization audits and cost-containment pilots in CA, FL, TX, and NY — the states with the largest financial footprint — before expanding programs elsewhere.

SPENDING DRIVERS

Is spending driven more by service volume or by pricing levels?

Volume Swings 3.7× Across States — Price Barely Moves



\$31-\$39

cost-per-service range across ALL 10 states — volume is the variable that moves.

Watch — Georgia: one of the highest markup ratios (4.12, second only to Texas's 4.26) despite the lowest total spend, showing pricing intensity can run independent of system scale.



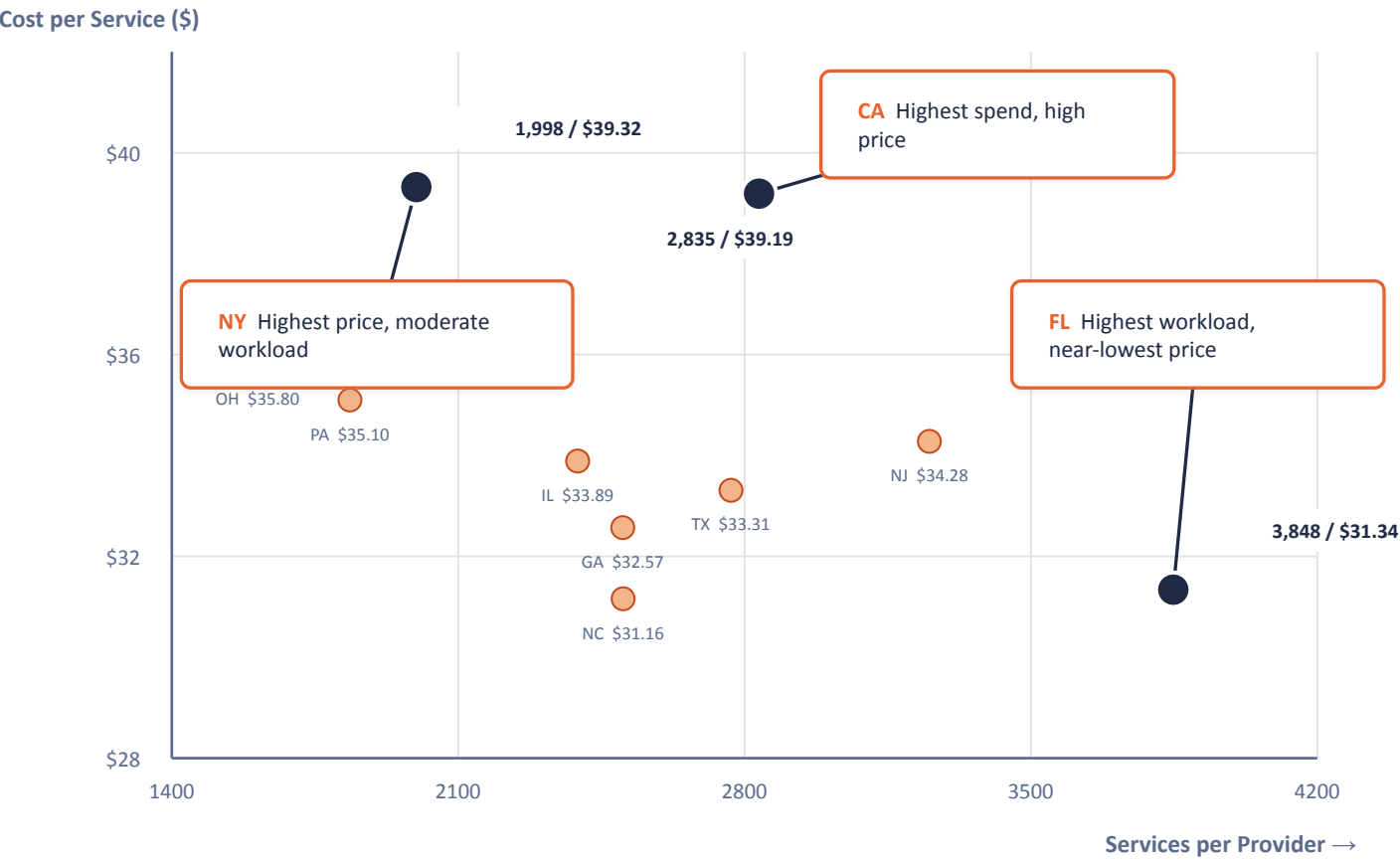
Recommendation: Shift cost-containment strategy toward utilization management and volume benchmarks — not price renegotiation.

PROVIDER CAPACITY & EFFICIENCY

How does provider capacity affect efficiency, and where is operational pressure highest?

Two Distinct Pressure Patterns, Not One

Workload vs. Pricing, by State



Markup ratio (e.g., TX, GA) does not consistently track either axis — pricing structure is a separate lever from both workload and unit cost.

WORKLOAD PRESSURE

FL · NJ · TX

High volume per provider, low-to-mid price. Action: expand provider capacity in these states.

COST-DRIVEN PRESSURE

NY · CA

High price without matching utilization. Action: review pricing and reimbursement structures.

FUTURE RESOURCE STRAIN

Which states show early indicators of future resource strain?

FL, CA, and NJ Already Show Structural Strain

FL	CA	NJ
OPERATIONAL STRAIN • Highest services/provider: 3,848 • 10.5% of the national service volume • Highest payments/provider: \$120.6K <i>Providers are saturating under workload.</i> Action: Expand provider capacity.	FINANCIAL CONCENTRATION • Highest total payments: \$93.2B • Near-highest cost/service: \$39.19 • High payments/provider: \$111.1K <i>Cost amplification at full system scale.</i> Action: Review pricing & reimbursement.	SUSTAINED OVERLOAD • Very high services/provider: 3,252 • High payments/provider: \$111.5K • Elevated markup ratio: 3.91 <i>Providers absorbing high, sustained demand.</i> Action: Add staffing / capacity support.



Recommendation: Begin proactive monitoring now for Nevada, Arizona, and Maryland — they show early signs of combined workload and cost intensity and risk reaching FL/CA/NJ-level strain without intervention.

What Leadership Should Take Away

WHAT WE FOUND

- Spending is heavily concentrated: CA, FL, TX, and NY together account for 34.5% of the entire \$853.6B national Medicare total — despite being just 4 of 51 states (all 10 states shown here make up 56% of the national total).
- Volume drives spending, not price — cost per service holds within \$31–\$39 nationally while service volume swings 3.7×.
- Two distinct efficiency problems exist: workload pressure (FL, NJ, TX) vs. cost-driven pressure (NY, CA) — they need different fixes.
- FL, CA, and NJ already show structural strain; NV, AZ, and MD are early-warning states.

RECOMMENDED ACTIONS

- 1 Control utilization in CA, FL, TX, and NY**
Set services-per-provider benchmarks — the real spending lever.
- 2 Stand up efficiency KPIs across all states**
Track provider productivity and utilization consistently.
- 3 Investigate pricing outliers (e.g., GA)**
High markup with lower spend may signal service-mix issues.
- 4 Monitor NV, AZ, and MD now**
Get ahead of strain before it becomes structural.

Bottom line: This is a utilization and capacity story, not a pricing story — act on volume and workload first.